

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Name: Patient 38

Age: 58 years old | Sex: Female

MRN: 000-TEST-38

Admission Date: [admission date] | Discharge Date: [discharge date]

Attending Physician: [physician name]

****PRIMARY DIAGNOSIS & COMORBIDITIES****

Primary: Total Hip Arthroplasty (THA), Right Hip — Severe Osteoarthritis with femoral head collapse

Secondary Diagnoses:

- Chronic Heart Failure (EF 35-40%, NYHA Class II-III)
- Obstructive Sleep Apnea (OSA) — on CPAP nightly
- Hypertension — controlled
- Type 2 Diabetes Mellitus — insulin-dependent
- Hyperlipidemia

****HOSPITAL COURSE & PROCEDURES****

Patient 38 was admitted electively for right total hip arthroplasty on [date]. Preoperative workup included EKG, CXR, labs, and cardiology clearance given significant cardiac history. Procedure completed under spinal anesthesia with epidural analgesia; cemented acetabular component and uncemented cementless femoral stem utilized. Estimated blood loss: 450 mL.

No intraoperative complications noted. Patient extubated successfully; transferred to ICU for continuous cardiac monitoring given CHF.

****Post-operative Course:****

POD 1: Patient alert and oriented x3, SpO2 95-98% on 2L NC, pain controlled with epidural infusion (morphine/bupivacaine). PT initiated bedside ROM exercises; TTWB status confirmed. DVT prophylaxis initiated (enoxaparin 40 mg SC daily).

POD 2–3: Epidural discontinued; transitioned to oral analgesia (see medications). Continued TTWB; PT advanced to ambulation with walker. Wound status stable, no signs of infection or hematoma.

POD 4 (Discharge): Afebrile, vital signs stable. Pain well-controlled. Patient demonstrates appropriate hip precautions and understanding of weight-bearing status.

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
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Acetaminophen	650 mg	Q 6h PO	4 weeks PRN
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Oxycodone	5 mg	Q 4h PO	2 weeks PRN pain
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Enoxaparin (Lovenox)	40 mg	SC daily	10 days total
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Metoprolol	25 mg	BID PO	Ongoing
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Lisinopril	10 mg	q.d. PO	Ongoing
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Insulin glargine	18 units	q.d. SC	Ongoing
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Atorvastatin	40 mg	q.d. PO	Ongoing
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****ACTIVITY RESTRICTIONS & WEIGHT-BEARING****

****Ambulation Status:**** TTWB (Toe-touch weight-bearing) right lower extremity for 6 weeks, then advance to WBAT (weight-bearing as tolerated) as tolerated per PT protocol.

****HIP PRECAUTIONS — STRICT ADHERENCE REQUIRED:****

- NO hip flexion >90 degrees (no bending at waist past 90°)
- NO internal rotation of operative hip
- NO crossing of legs
- Avoid low chairs; use raised toilet seat
- Use long-handled reacher for objects on ground
- Sleep with pillow between knees

****WOUND CARE****

Incision: Right lateral hip, approximately 4 inches, closed with staples. Wound clean, dry, intact at discharge. Sta